Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

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INFORMED CONSENT FORM

Temporal Lift

Lifting of the outer brow and eye corner through the temples (lateral brow lift)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left ☐ Both sides

Incision site: ☐ Within the scalp (temple) ☐ At the hairline ☐ To be decided during surgery _____

Fixation method: ☐ Permanent stitches ☐ Absorbable device ☐ Other _____

Procedures to be performed at the same time: ☐ Upper blepharoplasty (separate consent) ☐ Fat injection ☐ None _____

Patient's concerns and expectations: _____

1 Your condition and the aim of the procedure

With age, the tissues of the forehead and temples relax; in particular, the **outer end of the eyebrow** and the **outer corner of the eye** descend. This can give a tired, sad or heavy look, the appearance of excess upper-eyelid skin and wrinkles at the corner of the eye. In some people this shape is also present from birth.

A temporal lift is an operation that **lifts the outer part of the eyebrow and the corner of the eye upwards and backwards** through small incisions within the hair of the temples. Its aim is to achieve a **fresher, more open look** in harmony with your face. On its own it does not correct the inner part of the brow, the whole forehead or excess eyelid skin; if needed, it is combined with other procedures.

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2 How is the procedure performed?

The operation is usually performed under general anaesthesia, or sedation with local anaesthetic, in an operating theatre. Beforehand, the position of your eyebrows and eye corners is assessed with you standing and sitting, photographs are taken, and the direction of lift is marked together with you.

1. **Incision:** On each side, incisions of about 3–5 cm are made within the hair of the temple or at the hairline. Hair is not shaved, or only a very narrow strip along the incision is trimmed.
2. **Releasing the tissues:** The surgeon works in the deep layer of the temple in a way that protects the forehead branch of the facial nerve. The connective tissues around the outer brow and eye corner are gently released so that they can be moved.
3. **Lifting and fixation:** The released tissues are lifted upwards and backwards in the planned direction and secured to the firm tissue layer of the temple with **permanent stitches** or **absorbable fixation devices**. The balance between the two sides is checked.
4. **Closure:** If needed, a small amount of excess scalp is removed. The incisions are closed with staples or stitches, sometimes a thin drain is placed, and a light head dressing is applied.

If upper eyelid surgery (blepharoplasty) or another procedure is to be performed at the same time, a **separate consent form** will be signed for it.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia or sedation + local anaesthesia	About 1–2 hours	Usually same day or one night in hospital

3 Expected benefits and likelihood of success

- A fresher, more open and younger look as the outer brow and eye corner are lifted
- Less heaviness and apparent excess skin over the outer upper eyelid
- No visible facial scar, as the incisions are hidden in the hair

Most suitably selected patients are satisfied with the result. In the first few weeks the brows may look slightly higher than planned; this softens as the tissues settle. The final result is seen within **3–6 months**. The lifted tissues may descend somewhat over time with ageing and gravity; how long the effect lasts varies between individuals and is usually **several years**. About **5–10 in every 100** patients may need a correction because of asymmetry or early loosening.

4 Other treatment options

- **Not having surgery:** As this is a cosmetic procedure, you always have the option not to have it.
- **Brow lift with botulinum toxin:** Can raise the outer brow by a few millimetres, temporarily (3–4 months).
- **Thread-lift techniques:** Performed with fewer incisions, but the effect is usually more limited and shorter-lasting.
- **Fillers to support the temple/brow:** May give temporary improvement where descent is due to volume loss; they have their own risks, such as blockage of a blood vessel.
- **Other brow-lift operations:** If the whole forehead and the inner brow have also descended, an endoscopic forehead lift or methods using an incision above the brow or at the hairline may be more suitable.
- **Upper blepharoplasty alone:** May be enough if the main problem is excess eyelid skin; it does not correct a low brow.

5 What happens if the procedure is not performed?

If the operation is not performed, the current appearance of your brows and eye corners will remain and natural ageing will continue. This does not pose a risk to your health.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time

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before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Blood tests and, if needed, other investigations are performed before surgery; if general anaesthesia is planned, you will be assessed by the anaesthetist.
- Do not eat or drink anything for **6–8 hours** before surgery (follow the anaesthetist's instructions).
- Wash your hair the night before or the morning of surgery; do not use hairspray, gel, make-up or contact lenses on the day. Cutting your hair is usually not necessary.
- Tell your doctor if you have had botulinum toxin in the forehead or brow area in the last **3–4 months**; it may affect the planning.
- It is recommended that someone stays with you for the first day after surgery.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	You will have tightness, a mild headache and swelling at the temples and forehead; pain is usually mild and controlled with painkillers. Swelling may move down towards the eyes; bruising, **if it occurs**, appears on the eyelids. Sleeping with your head raised and applying cold compresses help to reduce swelling. If a drain was placed, it is usually removed the next day.
Week 1	You can usually wash your hair gently with a mild shampoo from **day 2–3**. The brows and eye corners may look higher and tighter than planned at this stage. Most patients return to daily activities and desk work within 5–7 days.
Days 7–14	Staples or stitches are removed. Numbness, itching and tenderness along the incision at the temples may continue.
Weeks 2–6	Swelling largely settles and the look becomes more natural. Strenuous exercise and tight headbands/helmets that press on the temples should be avoided for at least **3–4 weeks**, and hair colouring and similar chemical treatments for at least **4–6 weeks**; use your hairdryer on a warm setting.
Months 3–6	The tissues settle and the final appearance is seen. Changes in scalp sensation largely recover; the scar fades.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

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The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Tightness, a feeling of pressure and mild headache at the temples and forehead
- Swelling of the forehead and around the eyes; bruising, **if it occurs**, settles within 1–2 weeks
- Brows looking higher than planned, or a surprised look, in the first few weeks
- Numbness, itching or tingling of the temples and scalp
- Tenderness and firmness along the incision

Common conditions (most are temporary or resolve with treatment)

- **Mild asymmetry** — Eyebrows are not perfectly symmetrical to begin with; slight differences between the two sides may remain after surgery or appear during healing.
- **Hair thinning along the incision** — Temporary hair shedding around the incision (shock loss) is common, and the hair usually grows back within **2–4 months**. The incision remains as a fine line covered by hair.
- **Prolonged numbness and itching** — Altered sensation of the scalp may last several months; itching during healing is normal.
- **Partial loosening** — Some settling of the lifted tissues is expected in the first months; for this reason a slight overcorrection may be made during surgery.

Uncommon conditions

- **Temporary weakness in raising the brow (forehead branch of the facial nerve)** — In about **1–3 in every 100** patients there may be temporary weakness in raising the brow or wrinkling the forehead on one side. This is usually due to stretching of the nerve or swelling and recovers on its own within **a few weeks to a few months**.
- **Permanent hair loss or a wide scar** — A permanent hairless area or widened scar along the incision may rarely develop; scars at the hairline in particular may be more noticeable. If needed, it can be improved with scar revision or hair transplantation.
- **Haematoma (collection of blood)** — In about **1–2 in every 100** patients blood may collect in the temple; it may need to be drained with a needle or a small procedure.
- **Infection** — Occurs in **fewer than 1 in 100** patients. It usually settles with antibiotics; rarely drainage or removal of a fixation stitch is needed.
- **Overcorrection: a "surprised" or overly slanted ("fox eye") look** — The outer brow or eye corner may appear lifted more than desired. This usually softens as the tissues settle in the first few weeks; if it persists, a correction may be needed.
- **Early descent or insufficient effect** — Because of loosening of the fixation or the nature of the tissues, the brow may descend earlier than expected; a repeat lift may be needed.
- **Problems related to fixation material** — A permanent stitch or device may be felt under the skin, may be tender or may rarely work its way through the skin; in that case it may need to be removed.

Rare or very rare, but potentially serious conditions

- **Permanent nerve damage** — Permanent damage to the nerve branch that moves the brow-raising muscle is **very rare** (far fewer than 1 in 100 patients). In this case the brow on that side does not lift and the forehead does not wrinkle; botulinum toxin to the opposite side or further surgery may be needed to reduce the asymmetry.
- **Permanent numbness or chronic pain** — Permanent loss of scalp sensation, prolonged pain due to nerve entrapment, or headaches may very rarely occur; further treatment may be needed.

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- **Impaired skin blood supply** — Especially in smokers, wound-healing problems at the incision edges may occur, leading to a wider scar and hair loss.
- **Difficulty closing the eyes and dry eye** — If upper blepharoplasty was performed in the same session, difficulty closing the eye fully and dry-eye symptoms may rarely occur; most are temporary.
- **Psychological dissatisfaction** — Despite a technically successful result, a person may find it difficult to adjust to their new look. Discussing expectations openly before surgery reduces this risk.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Rapidly increasing, tense and painful swelling** of the temple or forehead
- **Bleeding that does not stop** from the incisions
- **Fever above 38.5 °C**, increasing redness, pain, warmth or pus at the incision sites
- **Severe headache** not relieved by painkillers, vomiting or neck stiffness
- **Blurred or double vision**, eye pain or being unable to close the eye
- Darkening, purple-black discolouration of the skin, or the wound opening up

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- For the first 1–2 weeks sleep on your back with your head raised; avoid sleeping face down or on your temples.
- For the first 3–4 weeks do not rub your brows or forehead, do not massage the temples, and avoid movements that pull your face downwards.

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- Postpone chemical and heat treatments such as hair colouring, perms and hair straighteners for at least **4–6 weeks**. Use your hairdryer on a warm setting.
- Avoid strenuous exercise, bending, lifting, saunas and Turkish baths for the first 3–4 weeks.
- If you have travelled from abroad, flying is usually safe after the first check-up, generally from day 3–5.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

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14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that a temporal lift raises the outer part of the brow and the corner of the eye, and that on its own it does not correct the whole forehead, the inner brow or excess eyelid skin.
- ☐ I understand that in the first few weeks the brows may look higher and tighter, that the lifted tissues may descend somewhat over time, and that the effect is not permanent.
- ☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.
- ☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

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15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.